

Prenatal Home Visit Protocol

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This protocol governs Wednesday morning prenatal home visit routes. All CNM home visits are conducted within the scope of a collaborative practice agreement with the designated backup OB/GYN group. This document is for internal use only and supplements - not replaces - the clinical judgment of the attending midwife.

Pre-Visit Preparation

Chart Review (night before)

- Confirm gestational age and EDC for each client on the route
- Review most recent vital signs, labs, and any outstanding concerns from the previous visit
- Note any high-risk flags: GDM, hypertensive disorders, malpresentation after 36 weeks, IUGR
- Confirm client phone number and address - notify clients of approximate arrival window (30-minute window)
- Review any consult notes or specialist communications received since the last visit

Clinical Bag Check (every on-call week)

Item	Qty	Check
Fetoscope / Doppler with gel	1 ea	■
Sterile exam gloves (M and L)	2 boxes	■
Non-sterile gloves	1 box	■
Calibrated BP cuff (manual)	1	■
Pulse oximeter	1	■
Thermometer with probe covers	1	■
Tape measure (fundal height)	1	■
Urine dipstick strips (glucose, protein, nitrites)	1 tube	■
Specimen collection cups (3)	3	■
GBS culture swabs (if applicable)	As needed	■
Patient education handouts (prenatal, postpartum, feeding)	3	■
Printed route sheet with client names and addresses	1	■

Visit Flow

The standard prenatal home visit runs 45-60 minutes per client. Scheduling allows 15 minutes of travel and buffer between appointments.

1. Arrival and greeting (5 minutes). Knock and identify yourself. Confirm client's name and gestational age verbally. Observe the home environment briefly - safety, cleanliness, and support systems matter for risk assessment. Wash hands before any clinical contact.

2. Maternal vitals (10 minutes). Obtain blood pressure in the seated position after 5 minutes of rest, pulse, oxygen saturation, and temperature if clinically indicated. Document all values. BP above 140/90 on two readings 4 hours apart triggers urgent evaluation protocol.

3. Fundal height (5 minutes). Measure in centimeters from the pubic symphysis to the fundal dome. Record and compare to prior measurement. Fundal height in centimeters should approximate gestational age in weeks ± 2 cm after 20 weeks. Discrepancy greater than 3 cm warrants ultrasound.

4. Fetal heart tones (5 minutes). Obtain Doppler FHT and document baseline rate (normal 110-160 bpm). Note variability if audible. Reassure client with verbal confirmation of fetal heartbeat.

5. Fetal presentation (from 34 weeks) (5 minutes). Perform Leopold maneuvers to assess fetal lie, presentation, and engagement. Document findings. If malpresentation at 36+ weeks, initiate referral for ECV evaluation.

6. Urine dipstick (3 minutes). Collect a clean-catch mid-stream specimen if indicated. Dip for protein, glucose, and nitrites. Protein 2+ or greater triggers further evaluation for preeclampsia. Positive nitrites with symptoms prompts culture and treatment.

7. Maternal wellbeing screen (10 minutes). Ask the Edinburgh Prenatal Depression Scale two-question screen at each visit after 28 weeks. Discuss nutrition, hydration, sleep, and activity. Screen for domestic violence using standard HITS protocol at least once per trimester.

8. Education and planning (10 minutes). Review birth plan if not yet completed. Discuss signs of labor (regular contractions, ruptured membranes, decreased fetal movement). Confirm hospital backup plan and transport arrangement. Provide relevant handout.

Urgent Referral Criteria

Any of the following findings during a home visit require immediate contact with the collaborating OB and, if clinically urgent, direct transport to the backup hospital. Do not leave the client unattended until a plan is in place.

- BP $\geq 160/110$ with symptoms (headache, visual changes, epigastric pain)
- BP $\geq 140/90$ on two readings with proteinuria 2+
- Fetal heart tones absent or persistently less than 100 bpm or greater than 180 bpm
- Suspected ruptured membranes with no labor at less than 37 weeks
- Vaginal bleeding beyond normal show
- Client reports absence of fetal movement for more than 12 hours
- Altered mental status, severe headache, or visual disturbance

Post-Visit Documentation

All visit findings must be documented in the clinical record within 4 hours of the visit. Documentation includes: vitals with timestamps, FHR and presentation, any abnormal findings and actions taken, education provided, and next visit scheduling. If a referral was initiated, document the time of contact with the OB and the plan communicated to the client.